

ASHAMitra — Clinical Modules (detailed)

Auto-generated from `assets/data/asha_engine.json` — engine `asha_cdss_engine_v2` **v2.6.0-guideline-aligned**. Companion docs: [11-step pipeline](#) · [sign-off rules](#) · [overview + backend](#).

How to read a rule type: *hard-stop* = one matching answer alone → RED · *combination* = all listed answers together → the shown band · *numeric* = a measured vital crossing the limit · *yellow* = a single smaller worry → YELLOW · *score* = points added per positive answer. Rules marked † are flagged `clinical_sign_off_pending` (live, but awaiting clinician sign-off — see [SIGNOFF_RULES.md](#)). Unsure about a medical word? See the [plain-words glossary](#) at the bottom.

Bands: ● home care · ● refer PHC within 24 h · ● refer FRU/SNCU/DH now (ambulance 108).

● In easy words

A **module is a checklist for one type of patient**. The app picks the right one (newborn, child, pregnant woman, etc.), asks a few yes/no questions, looks at a few measurements, and gives a colour: ● fine · ● see a clinic · ● emergency.

Each checklist mixes five kinds of rules: - **Hard-stop** — one dangerous "yes" alone → ● RED (no debate). - **Combination** — two worries *together* that are dangerous as a pair. - **Numeric** — a *measured* number crossing a safe limit (e.g. oxygen below 90%). - **Yellow** — a smaller worry → ● see a clinic. - **Score** — small worries add up points; enough points raises the level.

Everything below is just those five ingredients filled in for each type of patient. The strictness changes by patient: a **newborn** turns almost every sign into ● RED (babies get sick fast), while **immunisation** and **development** never go past ● YELLOW.

🍼 Newborn Checklist (0–28 days) — newborn

নবজাতক চেকআপ (০-২৮ দিন)

Who: Baby 0–28 days

Purpose: Built around **PSBI (Possible Serious Bacterial Infection)**. Newborns deteriorate within hours, so the list is the strictest in the app — almost every danger sign is an immediate RED referral to a newborn special-care unit (SNCU) or first-referral unit (FRU).

● **In easy words:** A baby less than one month old. Newborns can go from fine to very sick in just hours, so nearly every warning sign here means "**rush to a hospital with newborn care now.**"

Questions (14)

ID	Question
n1	Is the baby unable to breastfeed?
n2	Does the baby have fever?
n3	Is breathing fast or difficult?
n4	Is the navel red, swollen, or has pus?
n5	Is the baby lethargic or not moving?
n6	Does the skin look yellow or bluish?
n7	Has the baby had convulsions or abnormal movements?
n8	Is the baby cold to touch / colder than normal (hypothermia)?
n9	Many skin pustules / pus-filled blisters, or pus draining from ear?
n10	Is the soft spot on the head (fontanelle) bulging?
n11	Has the baby NOT passed stool in the first 24 h, or NOT passed urine in the first 48 h?
n12	Does the baby have loose stools / diarrhoea?

ID	Question
n13	Are the baby's eyes red, swollen, or draining pus?
n14	Does the baby have a visible congenital defect (e.g. cleft lip/palate, abnormal limb)?

Hard-stop rules → RED (13)

Rule	Trigger (answer = yes)	Refer to	Suspected condition
NB-001	Is the baby unable to breastfeed?	SNCU / FRU immediately	Possible neonatal sepsis
NB-002	Does the baby have fever?	SNCU / FRU immediately	Neonatal fever / sepsis
NB-003	Is breathing fast or difficult?	SNCU / FRU immediately	Respiratory distress
NB-004	Is the navel red, swollen, or has pus?	FRU / DH immediately	Omphalitis
NB-005	Is the baby lethargic or not moving?	SNCU / FRU immediately	Possible neonatal sepsis
NB-006	Does the skin look yellow or bluish?	SNCU / FRU immediately	Jaundice / Cyanosis
NB-007	Has the baby had convulsions or abnormal movements?	SNCU / FRU immediately	Neonatal seizures, Possible sepsis / meningitis
NB-008 †	Is the baby cold to touch / colder than normal (hypothermia)?	SNCU / FRU immediately	Neonatal hypothermia
NB-010 †	Many skin pustules / pus-filled blisters, or pus draining from ear?	FRU / DH immediately	Local bacterial infection (PSBI)
NB-011 †	Is the soft spot on the head (fontanelle) bulging?	SNCU / FRU immediately	Meningitis
NB-012	Has the baby NOT passed stool in the first 24 h, or NOT passed urine in the first 48 h?	FRU / SNCU immediately	Intestinal/urinary obstruction or serious neonatal illness
NB-013	Does the baby have loose stools / diarrhoea?	FRU / SNCU immediately	Neonatal diarrhoea / dehydration
NB-014	Are the baby's eyes red, swollen, or draining pus?	FRU / DH immediately	Ophthalmia neonatorum

Combination rules (5)

Rule	If ALL of these are yes	Band	Suspected condition
NB-COMB-001	n2 (Does the baby have fever?) + n3 (Is breathing fast or difficu)	 RED	Severe PSBI
NB-COMB-002 †	n6 (Does the skin look yellow or) + n1 (Is the baby unable to breast)	 RED	Severe jaundice / kernicterus risk
NB-COMB-003 †	n5 (Is the baby lethargic or not) + n3 (Is breathing fast or difficu)	 RED	Severe neonatal sepsis
NB-COMB-004 †	n4 (Is the navel red, swollen, o) + n2 (Does the baby have fever?)	 RED	Omphalitis with systemic sepsis
NB-COMB-005 †	n1 (Is the baby unable to breast) + n2 (Does the baby have fever?)	 RED	Possible serious bacterial infection (PSBI)

Numeric (vital) rules (9)

Rule	Vital	Condition	Band	Meaning
NB-VITAL-001	spo2	< 90	 RED	SpO2 < 90% — critical hypoxia.

Rule	Vital	Condition	Band	Meaning
NB-VITAL-002	respiratory_rate	≥ 60	● RED	RR ≥ 60/min — severe tachypnoea.
NB-VITAL-003	temperature_c	≥ 37.5	● RED	Temp ≥ 37.5°C — danger sign in newborn.
NB-VITAL-004	weight_kg	< 1.5	● RED	Weight < 1.5 kg — VLBW. Start KMC, refer SNCU immediately.
NB-VITAL-005 †	temperature_c	< 35.5	● RED	Temp < 35.5°C — hypothermia. Start kangaroo mother care, refer SNCU.
NB-VITAL-006 †	heart_rate	> 180	● RED	HR > 180/min — severe tachycardia. Refer SNCU.
NB-VITAL-007 †	weight_kg	1.8–2.5	● YELLOW	Weight 1.8–2.5 kg — LBW. KMC, breastfeed every 2 h.
NB-VITAL-008 †	weight_kg	< 1.8	● RED	Weight < 1.8 kg — HBNC referral threshold. KMC + expressed breast milk + refer SNCU.
NB-VITAL-009 †	temperature_c	35.5–36.4	● YELLOW	Temp 35.5–36.4°C — cold stress. KMC, warm the room, recheck in 1 hour.

Yellow rules → ● YELLOW (1)

Rule	Trigger	Refer to	Suspected
NB-Y-001	Does the baby have a visible congenital defect (e.g. cleft lip/palate, abnormal limb)?	PHC / DH for assessment	Congenital anomaly

Risk score

Weights: n1 = 3 · n2 = 2 · n3 = 3 · n4 = 2 · n5 = 3 · n6 = 2 · n7 = 3 · n8 = 3 · n9 = 2 · n10 = 3

Bands by score: ● GREEN 0–2 · ● YELLOW 3–5 · ● RED 6–100

Child Health Check (2 months–5 years) — child

শিশু স্বাস্থ্য যাচাই (২ মাস–৫ বছর)

Who: Child 2 months–5 years

Purpose: Follows the **IMNCI** approach. Screens for the leading under-5 killers: pneumonia, diarrhoea/dehydration, prolonged fever (malaria/dengue/typhoid), severe acute malnutrition, and meningitis/sepsis danger signs.

● **In easy words:** A small child (2 months–5 years). Looks for the things that most often kill young children — chest infection, dehydration from diarrhoea, long fevers, and severe thinness — plus emergency signs like fits.

Questions (13)

ID	Question
c1	Has fever lasted more than 5 days?
c2	Is there cough or breathing difficulty?
c3	Is there diarrhoea or repeated vomiting?
c4	Is the child completely refusing to eat?
c5	Signs of dehydration — sunken eyes, dry lips?
c6	Is weight much less than expected for age?
c7	Has the child had convulsions or fits?
c8	Is the child abnormally sleepy, lethargic, or unconscious?

ID	Question
c9	Does the child vomit everything?
c10	Stiff neck, light sensitivity, or severe headache with fever?
c11	Is there blood in the stool (dysentery)?
c12	Are the palms or inner eyelids very pale (anaemia)?
c13	Lower chest indrawing, or stridor at rest?

Hard-stop rules → RED (8)

Rule	Trigger (answer = yes)	Refer to	Suspected condition
CH-001	Has fever lasted more than 5 days?	PHC / DH same day	Prolonged fever — malaria / typhoid / dengue
CH-005	Signs of dehydration — sunken eyes, dry lips?	FRU immediately	Severe dehydration
CH-007	Has the child had convulsions or fits?	FRU / DH immediately	Seizure, Meningitis / cerebral malaria
CH-008	Is the child abnormally sleepy, lethargic, or unconscious?	FRU / DH immediately	Severe systemic illness, Shock / sepsis
CH-009 †	Does the child vomit everything?	FRU / DH immediately	Unable to retain feeds, Possible serious illness
CH-010 †	Stiff neck, light sensitivity, or severe headache with fever?	FRU / DH immediately	Meningitis, Encephalitis
CH-004 †	Is the child completely refusing to eat?	FRU / DH immediately	Very severe disease, Feeding refusal
CH-013 †	Lower chest indrawing, or stridor at rest?	FRU / DH immediately	Severe pneumonia / very severe disease

Combination rules (4)

Rule	If ALL of these are yes	Band	Suspected condition
CH-COMB-001	c2 (Is there cough or breathing) + c5 (Signs of dehydration — sunke)	 RED	Severe pneumonia with dehydration
CH-COMB-002 †	c1 (Has fever lasted more than 5) + c2 (Is there cough or breathing)	 RED	Pneumonia, Malaria
CH-COMB-003 †	c3 (Is there diarrhoea or repeat) + c4 (Is the child completely refu)	 RED	Severe dehydration with feeding refusal
CH-COMB-004 †	c3 (Is there diarrhoea or repeat) + c5 (Signs of dehydration — sunke)	 RED	Severe dehydration (IMNCI Plan C)

Numeric (vital) rules (10)

Rule	Vital	Condition	Band	Meaning
CH-VITAL-001	spo2	< 90	 RED	SpO2 < 90% — critical hypoxia.
CH-VITAL-002	muac_cm	< 11.5	 RED	MUAC < 11.5 cm = SAM. Refer NRC.
CH-VITAL-003	muac_cm	11.5–12.5	 YELLOW	MUAC 11.5–12.5 cm = MAM. Refer ICDS.
CH-VITAL-004 †	respiratory_rate	≥ 50	 RED	RR ≥ 50/min (2 mo–1 y) = severe pneumonia. Refer FRU.
CH-VITAL-005a †	temperature_c	≥ 40	 RED	Temp ≥ 40°C = hyperpyrexia. Refer FRU, test for malaria.
CH-VITAL-005b †	temperature_c	39–40	 YELLOW	Temp 39–40°C = high fever. Refer PHC, give paracetamol.
CH-VITAL-006 †	weight_for_age_z	< -3	 RED	WFA Z-score < -3 = severe underweight. Refer NRC.
CH-VITAL-006b †	weight_for_age_z	-3--2	 YELLOW	WFA Z-score -3 to -2 = moderate underweight (MAM). Refer ICDS/Anganwadi for supplementary nutrition, weigh monthly. (MCP card: yellow zone, pg 28–31)

Rule	Vital	Condition	Band	Meaning
CH-VITAL-007 †	respiratory_rate	≥ 40	YELLOW	RR ≥ 40/min (child 1–5 y) = possible pneumonia. Refer PHC within 24 h. Chest indrawing = RED.
CH-VITAL-008 †	spo2	90–94	YELLOW	SpO2 90–94% = moderate hypoxia. Keep the child calm and upright, refer to PHC promptly.

Yellow rules →  **YELLOW (5)**

Rule	Trigger	Refer to	Suspected
CH-002	Is there cough or breathing difficulty?	PHC within 24 h	Pneumonia
CH-003	Is there diarrhoea or repeated vomiting?	PHC within 24 h	Diarrhoea / vomiting
CH-006	Is weight much less than expected for age?	ICDS / NRC if SAM	Malnutrition
CH-011 †	Is there blood in the stool (dysentery)?	PHC within 24 h	Dysentery
CH-012 †	Are the palms or inner eyelids very pale (anaemia)?	PHC for Hb check	Anaemia

Risk score

Weights: c1 = 3 · c2 = 2 · c3 = 2 · c4 = 3 · c5 = 3 · c6 = 2 · c7 = 3 · c8 = 3 · c9 = 3 · c10 = 3 · c11 = 2 · c12 = 2 · c13 = 4

Bands by score:  GREEN 0–2 ·  YELLOW 3–5 ·  RED 6–100

Pregnant Mother Checkup (ANC) — pregnancy

গর্ভবতী মায়ের চেকআপ

Who: Pregnant woman (female)

Purpose: **Antenatal (ANC)** screening. Focus on the big maternal killers: hypertensive disease (pre-eclampsia/eclampsia), antepartum haemorrhage, severe anaemia, sepsis/malaria, and reduced fetal movement.

 **In easy words:** A pregnant woman's check-up. Watches mainly for **high blood pressure** (which can lead to fits), **bleeding, very low blood (anaemia)**, infection, and the **baby moving less**.

Questions (14)

ID	Question
p1	Is blood pressure high?
p2	Is there swelling of legs or face?
p3	Is there bleeding or severe abdominal pain?
p4	Has the baby's movement reduced?
p5	Has ANC checkup been missed for 3+ months?
p6	Is there blurred vision?
p7	Has there been a convulsion or fit during pregnancy?
p8	Very pale, weak, or breathless on slight exertion (severe anaemia)?
p9	Fever, especially with chills/rigors?
p10	Sudden gush or continuous leaking of fluid (PROM)?
p11	Is there a headache?
p11d	Is the headache very severe, lasting >2 days, or worsening?
p12	Is there dizziness or weakness?
p9r	Fever WITH chills or rigors?

Hard-stop rules → RED (8)

Rule	Trigger (answer = yes)	Refer to	Suspected condition
ANC-001	Is blood pressure high?	FRU / DH immediately	Pre-eclampsia / Eclampsia
ANC-003	Is there bleeding or severe abdominal pain?	FRU / DH immediately	Antepartum haemorrhage
ANC-004	Has the baby's movement reduced?	FRU / DH same day	Reduced fetal movement
ANC-006	Is there blurred vision?	FRU / DH immediately	Imminent eclampsia
ANC-007	Has there been a convulsion or fit during pregnancy?	FRU / DH immediately	Eclampsia
ANC-008 †	Very pale, weak, or breathless on slight exertion (severe anaemia)?	FRU / DH	Severe anaemia in pregnancy
ANC-010 †	Sudden gush or continuous leaking of fluid (PROM)?	FRU / DH	Premature rupture of membranes
ANC-009R †	Fever WITH chills or rigors?	FRU / DH immediately by 108	Malaria, Maternal sepsis / chorioamnionitis

Combination rules (6)

Rule	If ALL of these are yes	Band	Suspected condition
ANC-COMB-001	p1 (Is blood pressure high?) + p2 (Is there swelling of legs or)	 RED	Pre-eclampsia
ANC-COMB-002	p6 (Is there blurred vision?) + p2 (Is there swelling of legs or)	 RED	Imminent eclampsia
ANC-COMB-003 †	p3 (Is there bleeding or severe) + p4 (Has the baby's movement redu)	 RED	Placental abruption, IUGR
ANC-COMB-004 †	p1 (Is blood pressure high?) + p6 (Is there blurred vision?)	 RED	Severe pre-eclampsia
ANC-COMB-005 †	p11 (Is there a headache?) + p11d (Is the headache very severe,)	 RED	Severe/persistent headache — pre-eclampsia
ANC-COMB-006 †	p11 (Is there a headache?) + p2 (Is there swelling of legs or)	 RED	Pre-eclampsia

Numeric (vital) rules (15)

Rule	Vital	Condition	Band	Meaning
ANC-VITAL-001	systolic_bp	≥ 140	 RED	Systolic BP ≥ 140 mmHg — pre-eclampsia.
ANC-VITAL-002	diastolic_bp	≥ 90	 RED	Diastolic BP ≥ 90 mmHg — pre-eclampsia.
ANC-VITAL-003	haemoglobin	< 7	 RED	Hb < 7 g/dL — severe anaemia.
ANC-VITAL-004	haemoglobin	7–10	 YELLOW	Hb 7–10 g/dL — moderate anaemia. Increase IFA.
ANC-VITAL-005 †	systolic_bp	≥ 160	 RED	Systolic BP ≥ 160 mmHg — severe hypertension. Lay on left side, call 108 and take to FRU now.
ANC-VITAL-006 †	urine_protein_plus	≥ 2	 RED	Urine protein ≥ 2+ = pre-eclampsia. Refer FRU.
ANC-VITAL-007 †	fundal_height_weeks_diff	> 4	 YELLOW	Fundal height >4 weeks discrepancy = IUGR or polyhydramnios suspicion.

Rule	Vital	Condition	Band	Meaning
ANC-VITAL-008 †	gestational_age_weeks	< 37	RED	Labour pain or leaking before 37 weeks = preterm labour. Refer FRU without cervical examination if pains/leaking present.
ANC-VITAL-009 †	labour_hours	> 12	RED	Labour pain > 12 h = obstructed labour risk. Refer FRU/DH immediately.
ANC-VITAL-010 †	leaking_hours_no_labour	> 12	RED	Leaking > 12 h without labour = PROM with chorioamnionitis risk. Refer FRU now.
ANC-VITAL-011 †	tsh_miu	> 10	RED	TSH > 10 mIU/L on report = thyroid problem. Refer to PHC/MO — do not start or change any medicine yourself.
ANC-VITAL-012 †	tsh_miu	2.5–10	YELLOW	TSH 2.5–10 mIU/L on report = mildly raised thyroid. Refer to PHC/MO; do not give medicine yourself.
ANC-VITAL-013 †	ogtt_2hr_mg_dl	≥ 140	YELLOW	High sugar on report (GDM). Refer to PHC/MO for GDM management. ASHA role: counsel balanced diet and ensure follow-up. Do not give medicine or insulin yourself.
ANC-VITAL-014 †	fhr_bpm	< 120	RED	FHR < 120 bpm = fetal distress. Position mother left lateral, refer FRU/DH immediately.
ANC-VITAL-015 †	fhr_bpm	> 160	RED	FHR > 160 bpm = fetal distress (maternal fever or chorioamnionitis). Refer FRU now.

Yellow rules → YELLOW (5)

Rule	Trigger	Refer to	Suspected
ANC-002	Is there swelling of legs or face?	PHC within 24 h	Oedema
ANC-005	Has ANC checkup been missed for 3+ months?	PHC within 7 days	Missed ANC
ANC-011 †	Is there a headache?	PHC within 24 h	Headache (assess for pre-eclampsia)
ANC-012 †	Is there dizziness or weakness?	PHC within 24 h	Dizziness (anaemia / hypotension)
ANC-009 †	Fever, especially with chills/rigors?	PHC within 24 h	Fever in pregnancy (rule out malaria/UTI/sepsis)

Risk score

Weights: p1 = 4 · p2 = 2 · p3 = 4 · p4 = 3 · p5 = 1 · p6 = 3 · p7 = 4 · p8 = 3 · p9 = 3 · p10 = 3 · p11 = 2 · p11d = 2 · p12 = 1 · p9r = 4

Bands by score: GREEN 0–2 · YELLOW 3–5 · RED 6–100

Postpartum Checkup (0–42 days) — delivery_pnc

প্রসব-পরবর্তী চেকআপ (০-৪২ দিন)

Who: Mother 0–42 days after delivery (female)

Purpose: Postnatal mother care. Focus on postpartum haemorrhage (PPH), puerperal sepsis, postpartum eclampsia, breathing difficulty, and a postpartum-depression screen.

In easy words: A mother in the ~6 weeks after delivery. Watches mainly for **heavy bleeding** and **infection (fever)** — the two biggest dangers after childbirth — plus her mood.

Questions (11)

ID	Question
pp1	Is there excessive bleeding or foul-smelling discharge?
pp2	Is there fever or chills?
pp3	Is there breast pain, swelling, or red marks?
pp4	Is there severe abdominal pain or suture problem?
pp5	Is there burning or difficulty urinating?
pp6	Is there extreme weakness or dizziness?
pp7	Has there been a convulsion or fit after delivery?
pp8	Is there difficulty breathing?
pp9	Persistent low mood, crying, sleep/appetite loss, or no interest in the baby?
pp6s	Fainting / collapsing, or unable to stand (severe weakness)?
pp10	Unable to control urine or stool (incontinence)?

Hard-stop rules → RED (5)

Rule	Trigger (answer = yes)	Refer to	Suspected condition
PNC-001 †	Is there excessive bleeding or foul-smelling discharge?	FRU / DH immediately	Postpartum haemorrhage, Puerperal sepsis
PNC-007	Has there been a convulsion or fit after delivery?	FRU / DH immediately	Postpartum eclampsia
PNC-008	Is there difficulty breathing?	FRU / DH immediately	Pulmonary embolism, Cardiac failure, Severe anaemia / sepsis
PNC-006S †	Fainting / collapsing, or unable to stand (severe weakness)?	FRU / DH via 108 now	Concealed PPH, Hypovolaemic shock
PNC-009	Unable to control urine or stool (incontinence)?	FRU / DH immediately	Obstetric fistula / sphincter injury

Combination rules (4)

Rule	If ALL of these are yes	Band	Suspected condition
PNC-COMB-001	pp1 (Is there excessive bleeding) + pp2 (Is there fever or chills?)	 RED	Puerperal sepsis
PNC-COMB-002	pp4 (Is there severe abdominal pa) + pp2 (Is there fever or chills?)	 RED	Peritonitis
PNC-COMB-003 †	pp1 (Is there excessive bleeding) + pp6 (Is there extreme weakness or)	 RED	Hypovolaemic shock
PNC-COMB-004 †	pp3 (Is there breast pain, swelli) + pp2 (Is there fever or chills?)	 RED	Breast abscess

Numeric (vital) rules (8)

Rule	Vital	Condition	Band	Meaning
PNC-VITAL-001	temperature_c	≥ 38	 RED	Temp ≥ 38°C postpartum = puerperal sepsis. Refer FRU.
PNC-VITAL-002	haemoglobin	< 7	 RED	Hb < 7 g/dL — refer FRU for transfusion.
PNC-VITAL-003	haemoglobin	7–10	 YELLOW	Hb 7–10 g/dL — moderate anaemia. Increase IFA.
PNC-VITAL-004 †	systolic_bp	≥ 140	 RED	Postpartum systolic BP ≥ 140 = postpartum pre-eclampsia. Lay on left side, call 108 and take to FRU now.

Rule	Vital	Condition	Band	Meaning
PNC-VITAL-005 †	heart_rate	> 110	YELLOW	HR > 110/min = sepsis or anaemia risk. Refer PHC for evaluation.
PNC-VITAL-006 †	pads_soaked_per_30min	≥ 2	RED	≥2 pads soaked in 30 min = PPH. Uterine massage, call 108.
PNC-VITAL-007 †	pads_per_day	> 5	RED	More than 5 pads per day (HBNC threshold) = excessive bleeding. PPH/sepsis risk — refer FRU now.
PNC-VITAL-008 †	temperature_c	≥ 38.9	RED	Temp ≥ 38.9°C postpartum = severe puerperal sepsis. Keep the mother warm and hydrated, do not give medicine yourself, call 108 and take to FRU now.

Yellow rules → YELLOW (6)

Rule	Trigger	Refer to	Suspected
PNC-002	Is there fever or chills?	PHC within 24 h	Puerperal fever
PNC-003	Is there breast pain, swelling, or red marks?	PHC within 24 h	Mastitis
PNC-004	Is there severe abdominal pain or suture problem?	PHC within 24 h	Wound infection
PNC-005	Is there burning or difficulty urinating?	PHC within 24 h	UTI
PNC-006	Is there extreme weakness or dizziness?	PHC for Hb check	Anaemia
PNC-009 †	Persistent low mood, crying, sleep/appetite loss, or no interest in the baby?	PHC / MO counselling	Postpartum depression

Risk score

Weights: pp1 = 4 · pp2 = 2 · pp3 = 1 · pp4 = 2 · pp5 = 1 · pp6 = 2 · pp7 = 4 · pp8 = 3 · pp9 = 1 · pp6s = 4

Bands by score: GREEN 0–2 · YELLOW 3–5 · RED 6–100

Immunization — UIP National Schedule — immunisation

টিকা মিস / ইমিউনাইজেশন

Who: Child 0–16 years (UIP schedule)

Purpose: Checks vaccine status against the national **UIP** schedule. No emergencies — flags missed/overdue doses for catch-up, defers if the child is currently ill, and routes a prior severe reaction (AEFI) to a doctor before the next dose.

In easy words: Checking a child's vaccines. **Never an emergency** — it just flags shots that are missed or late so they can be caught up, and pauses if the child is sick.

Questions (6)

ID	Question
im1	What is the child's age in months?
im2	Which vaccine was missed?
im3	How many days ago was the vaccine due?
im4	Does the child have any illness now?
im5	Has the booster dose been missed for child aged 1–5 years?
im6	Was there a severe reaction after a previous vaccine dose (AEFI)?

Yellow rules → YELLOW (6)

Rule	Trigger	Refer to	Suspected
IMM-001	What is the child's age in months?	Nearest immunisation session within 7 days	Missed primary vaccines
IMM-002	Which vaccine was missed?	Nearest immunisation session within 7 days	Missed vaccine
IMM-003	Has the booster dose been missed for child aged 1–5 years?	Next immunisation session	Missed booster
IMM-004	Does the child have any illness now?	Reschedule within 7 days of recovery	Illness at vaccination
IMM-005	How many days ago was the vaccine due?	PHC catch-up session	Long vaccine gap
IMM-006 †	Was there a severe reaction after a previous vaccine dose (AEFI)?	Refer MO before next dose	AEFI history

Risk score

Weights: im4 = 1 · im5 = 1 · im6 = 1

Bands by score:  GREEN 0–0 ·  YELLOW 1–100 ·  RED undefined–undefined

Child Development Screening (2 months–3 years) — development

শিশু বিকাশ যাচাই (২ মাস–৩ বছর)

Who: Child 2 months–3 years

Purpose: Early **child-development screening** using the MCP-card milestones. Flags a missed milestone for a development assessment at a DEIC / health centre.

 **In easy words:** Checking whether a small child is growing and learning on time — sitting, walking, talking. A missed milestone means "send for a check-up," not an emergency.

Questions (2)

ID	Question
d1	Which age band is the child in?
d2	Is ANY ONE of the danger signs listed for this age present? (check the list below)

Yellow rules → YELLOW (1)

Rule	Trigger	Refer to	Suspected
DEV-001	Is ANY ONE of the danger signs listed for this age present? (check the list below)	DEIC / নিকটতম স্বাস্থ্যকেন্দ্র — বিকাশ মূল্যায়ন	Developmental delay

Risk score

Weights: d2 = 3

Bands by score:  GREEN 0–0 ·  YELLOW 1–100 ·  RED undefined–undefined

Emergency — Global Hard-Stop Engine — emergency

জরুরি অবস্থা যাচাই

Who: Any patient (cross-cutting)

Purpose: A universal "is this a crisis now?" sweep usable for any patient — and **re-run in the background against every other module** so a life-threatening sign is never missed. Any positive = RED, reach FRU/DH within 30 minutes.

● In easy words: A quick **"is this a life-threatening crisis right now?"** check for anyone — heavy bleeding, fits, can't breathe, poisoning, snakebite, shock. Any one means **rush to a hospital within 30 minutes**.

Questions (8)

ID	Question
e1	Is there excessive bleeding?
e2	Has there been convulsion or loss of consciousness?
e3	Is there severe difficulty breathing?
e4	Is the patient unresponsive or unconscious?
e5	Snakebite, venomous sting, or animal bite?
e6	Poison, pesticide, or overdose ingested?
e7	Cold clammy skin, very weak pulse, extreme listlessness (shock)?
e8	Major injury, bleeding from a large wound, or severe burn?

Hard-stop rules → ● RED (8)

Rule	Trigger (answer = yes)	Refer to	Suspected condition
EM-001	Is there excessive bleeding?	FRU / DH ≤ 30 min	Haemorrhage
EM-002	Has there been convulsion or loss of consciousness?	FRU / DH immediately	Eclampsia / Seizure
EM-003	Is there severe difficulty breathing?	FRU / DH ≤ 30 min	Respiratory failure
EM-004	Is the patient unresponsive or unconscious?	FRU / DH ≤ 30 min	Unconsciousness
EM-006 †	Snakebite, venomous sting, or animal bite?	FRU / DH ≤ 30 min	Envenomation, Rabies risk
EM-007 †	Poison, pesticide, or overdose ingested?	FRU / DH ≤ 30 min	Poisoning / pesticide ingestion
EM-008 †	Cold clammy skin, very weak pulse, extreme listlessness (shock)?	FRU / DH ≤ 30 min	Shock / circulatory collapse
EM-009 †	Major injury, bleeding from a large wound, or severe burn?	FRU / DH ≤ 30 min	Major trauma, Severe burn

Risk score

Weights: e1 = 4 · e2 = 4 · e3 = 4 · e4 = 4 · e5 = 4 · e6 = 4 · e7 = 4 · e8 = 4

Bands by score: ● GREEN 0–0 · ● YELLOW undefined–undefined · ● RED 1–100

Glossary (medical words in plain language)

Term	In easy words
PSBI	Possible Serious Bacterial Infection — a dangerous infection in a newborn that can turn deadly fast.
IMNCI	The standard government method for checking sick young children.
ANC	Antenatal care — pregnancy check-ups <i>before</i> birth.
PNC	Postnatal care — mother/baby check-ups <i>after</i> birth.
Pre-eclampsia / Eclampsia	Dangerous high blood pressure in pregnancy; eclampsia is when it causes fits.
APH	Antepartum haemorrhage — bleeding during late pregnancy.
PPH	Postpartum haemorrhage — heavy bleeding after delivery (the #1 killer of new mothers).
PROM	Premature rupture of membranes — the water bag breaks early.
Puerperal sepsis	A serious infection in the mother after delivery.
Omphalitis	Infection of a newborn's belly-button (umbilical) area.

Term	In easy words
Kernicterus	Brain damage from very severe newborn jaundice.
Jaundice	Yellow colour of skin/eyes.
Cyanosis	Bluish skin/lips from low oxygen.
Hypothermia	Body too cold.
Fontanelle	The soft spot on a baby's head; bulging can mean brain infection.
Sepsis	Infection that has spread through the body — life-threatening.
MUAC	Mid-Upper-Arm Circumference — a coloured tape around the upper arm to check for malnutrition.
SAM / MAM	Severe / Moderate Acute Malnutrition (very thin / thin for the child's size).
Anaemia	Low blood (low haemoglobin) — causes weakness and danger in pregnancy.
SpO₂ / hypoxia	Oxygen level in the blood; hypoxia = too little oxygen.
Tachypnoea	Breathing too fast.
Tachycardia	Heart beating too fast.
Hyperpyrexia	Very high fever ($\geq 40^{\circ}\text{C}$).
GDM	Gestational diabetes — high blood sugar during pregnancy.
TSH	A thyroid blood test; abnormal values need a doctor.
FHR	Fetal heart rate — the baby's heartbeat.
Fundal height	Belly height of the womb, used to estimate the baby's growth.
IUGR	The baby growing slower/smaller than expected.
Dysentery	Diarrhoea with blood.
Mastitis	Painful, infected breast (while breastfeeding).
AEFI	A bad reaction after a vaccine.
UIP	The national vaccination schedule.
ORS	Oral Rehydration Solution — clean water + sugar + salt to treat dehydration.
KMC	Kangaroo Mother Care — keeping a small baby skin-to-skin on the chest to stay warm.
PHC	Primary Health Centre — the nearest small government clinic.
CHC	Community Health Centre — a bigger clinic above the PHC.
FRU	First Referral Unit — a hospital that can do emergencies/C-sections/transfusions.
SNCU	Special Newborn Care Unit — a newborn ICU.
DH	District Hospital — the big district-level hospital.
DEIC	District Early Intervention Centre — for child-development problems.
HBNC / HBYC	Home-Based Newborn / Young-Child Care — the ASHA's scheduled home visits.